



Laboratory Investigation Report

Patient Name	: Sharda	Centre	: 1534 - Arctern Healthcare Noida
Age/Gender	: 27 Y 0 M 0 D /F	OP/IP No	: OP/98687
Max ID/Mobile	: 441362/8752005200	Collection Date/Time	: 24/Aug/2021 02:28PM
Lab ID	: 1266082100566	Receiving Date	: 24/Aug/2021
Ref Doctor	: Dr.Dr. Richika Sahay Shukla	Reporting Date	: 24/Aug/2021

Clinical Pathology

Test Name	Result	Unit	Bio Ref Interval
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Urine Pregnancy Test*

Urine Pregnancy Test Immunochromatography	Negative		
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Limitations

The ehCG One Step Pregnancy Test Device (Urine) is a preliminary qualitative test, therefore, neither the quantitative value nor the rate of increase in hCG can be determined by this test.

Very dilute urine specimen, as indicated by a low specific gravity, may not contain representative levels of hCG. If pregnancy is still suspected, a first morning urine specimen should be collected 48 hours later and tested.

Very Low levels of hCG (less than 50miU/mL) are present in urine specimens shortly after implantation. However, because a significant number of first trimester pregnancies terminate for natural reasons, a test result that is weakly positive should be confirmed by retesting with a first morning urine specimen collected 48 hours later.

This test may produce false positive results. A number of conditions other than pregnancy including trophoblastic disease and certain non-trophoblastic neoplasms including testicular tumors, prostate cancer, breast cancer, and lung cancer, cause elevated levels of hCG. Therefore, the presence of hCG in urine should not be used to diagnose pregnancy unless these conditions have been ruled out.

This test may produce false negative results. False negative results may occur when the levels of hCG are below the sensitivity level of the test. When pregnancy is still suspected, a first morning urine specimen should be collected 48 hours later and tested. In case pregnancy is suspected and the test continues to produce negative results, see a physician for further diagnosis.

This test provides a presumptive diagnosis for pregnancy. A confirmed pregnancy diagnosis should only be made by a physician after all clinical and laboratory findings have been evaluated.

Kindly correlate with clinical findings

*** End Of Report ***



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SIN No: B2B899503, Test Performed at : 896 - Motherland Hospital, Motherland hospital Sec-119

Max Lab - A Division of Max Healthcare Institute Ltd. Floor, Powerstation by Oyo Pioneer Square, Sector 62, Gurugram, Haryana 122002, 8752 Max Super Specialty Hospital, Saket (West Block)

1, Press Enclave Road, Saket, New Delhi - 110 017, Phone: +91-11-6611 5050.

CIN No.: L72200MH2001PLC322854

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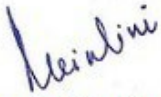
Conditions of Reporting: 1. The tests are carried out in the lab with the presumption that the specimen belongs to the patient name as identified in the bill/test request form. 2. The test results relate specifically to the sample received in the lab and are presumed to have been generated and transported per specific instructions given by the physicians/laboratory. 3. The reported results are for the information and interpretation by the referring doctor only. 4. Some tests are referred to other laboratories to provide a wider test menu to the customer. 5. Max Healthcare shall in no event be liable for accidental damages loss, or destruction of specimen which is not attributable to any direct and mala fide act or omission of Max Healthcare or its employees. Liability of Max Healthcare for deficiency of services, or other errors and omissions shall be limited to fee paid by the patient for the relevant laboratory services.

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Dr. Mrinalini Priyadarshini
DCP, DNB (Path.)
Consultant Pathologist



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Immunoassay

Test Name	Result	Unit	Bio Ref Interval
Anti Mullerian Hormone, Serum*			
Anti Mullerian Hormone (AMH) CLIA	23.55	ng/mL	0.17 - 7.37

Ref Range Interpretation :

Anti-Mullerian Hormone (AMH) is a hormone secreted by cells in developing egg sacs (follicles). The level of AMH in blood is generally a good indicator of ovarian reserve.
Low AMH levels are considered to be an indicator of a **low ovarian reserve**, i.e. few remaining follicles. AMH levels decline with age, and in younger women this may be a sign of premature loss of fertility
AMH does not change during menstrual cycle, so the blood sample can be taken at any time of the month - even while using oral contraception.
AMH level for a fertile woman is 1.0-4.0 ng/ml

In males AMH is secreted by immature Sertoli cells (SC) and is responsible for the regression of Müllerian ducts in the male fetus as part of the sexual differentiation process. AMH is also involved in testicular development and function.

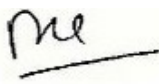
AMH level ng/ml	Effects for fertility treatment
<0.4	Very low value. Very few eggs at stimulation. Pregnancy chances significantly low.
0.4 – 1.0	Low value. Treatment may be possible.
1.0 – 3.5	Normal value. Good possibility of treatment.
>3.5	Suggestive of ovarian hyperstimulation syndrome / PCOS

Note :- Optimal ovarian reserve values range between 2 - 6 ng/mL in reproductive age group

Kindly correlate with clinical findings

*** End Of Report ***


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Principal Director-
Max Lab & Blood Bank Services


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Associate Director &
Manager Quality


Dr. Nitin Dayal, M.D.
Principal Consultant & Head,
Haematopathology



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